FRONT

BACK

PATIENT INFORMATION LEAFLET

Device Type: PMMA Intraocular Lenses

Introduction
Cataract is a condition that becomes more common as we age. A cataract forms over time as the eye's natural lens stiffens and gradually becomes opaque. People with cataracts experience 'cloudy' vision. Color become less vivid over time, and there is often increased sensitivity to light. For example, someone with cataract will likely experience an increased amount of glare from oncoming car headlights at night. Without treatment, cataract will eventually block light passing through the eye and cause blindness.

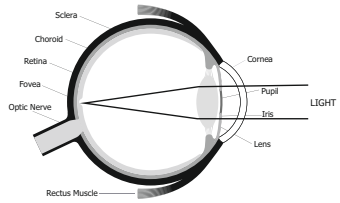


Fig. 1: The anatomy of the eye

How does cataract impact your vision?
A cataract affects the eye's natural lens, which is called the crystalline lens. Normally, this lens helps focus light onto the back of the eye (the retina) so we can see clearly. The lens is flexible and changes shape to help us focus on things both near and far. When we look at something close, the lens curves; when we look at something far away, it flattens. This ability of the lens to adjust for different distances is called "accommodation."

As we get older, the lens becomes less flexible, making it harder to focus on close objects—a condition called presbyopia, which can make reading small print difficult. A cataract adds another problem: the lens, which is normally clear, gradually becomes cloudy. This makes vision blurry over time. When cataracts start to affect vision, the cloudy natural lens needs to be removed and replaced with an artificial lens through surgery.

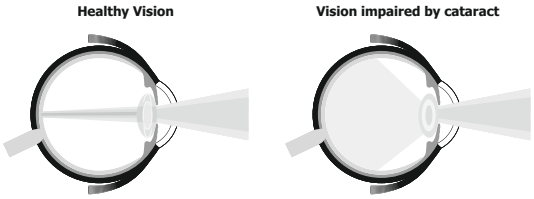


Fig. 2: Vision simulation without and with cataract

What is an Intraocular Lens (IOL)?
An intraocular lens (or IOL) is a small, clear artificial lens that is surgically placed inside your eye during cataract surgery to replace your natural crystalline lens when it becomes cloudy. Normally, your eye's natural lens focuses light onto the retina (the back wall of the eye) to produce sharp pictures. When a cataract develops, the natural lens turns cloudy, hazy, or yellowish, making your vision feel blurred or dim. During cataract surgery, the surgeon removes this cloudy lens and places an artificial intraocular lens in its place. The IOL stays inside your eye permanently.

There have been significant advancements in materials and designs for intraocular lens. We offer Intraocular lenses of different material types which includes: polymethyl methacrylate (PMMA), acrylic materials such as hydrophobic (repelling water) acrylic and hydrophilic (absorbing water) acrylic. PMMA is a rigid, non-foldable IOL known for its exceptional durability, long-term optical clarity, and proven clinical safety track record.



Fig. 3: Representative image of an Artificial Intraocular Lens

PMMA stands for polymethyl methacrylate. It is a smooth, high-purity, rigid clinical polymer that has been safely used in eye surgeries worldwide for decades due to its exceptional optical clarity and biocompatibility.
The Caregroup Single-Piece PMMA IOL is engineered to remain permanently inside your eye without degrading or breaking down over time. Your eye surgeon chooses the precise lens power based on specialized pre-operative measurements of your eye shape and length.

Different types and performance of the IOL
There are different types of lenses currently available. Your eye doctor will discuss which type of lens is best for you. Treatment options other than new permanent lens include glasses with thick lenses and contact lenses. Discuss potential risks and benefits of your treatment options with your eye doctor.

■ **Monofocal IOLs**
Monofocal IOLs restore clear distance vision. This means that you should see well when you go to a ball game or read distant signs. You will probably need glasses for near vision tasks such as reading a book, writing, or doing crafts. You may also need glasses for intermediate vision activities such as working on a computer, putting on make-up, or shaving.

■ **Multifocal IOLs**
All IOLs generally are designed for distance vision. However, multifocal IOLs are also designed to improve near and intermediate vision compared to a monofocal lens. This means that you should see well whether you go to a ball game or read a book. Multifocal lenses have different powers for near vision, based on your lifestyle needs. Overall, you may wear glasses less often for daily tasks. However, the sharpness of your vision may also decrease compared to a monofocal lens. This is especially the case under poor visibility conditions such as dim light or fog. You may also notice more halos (rings around light) or glare (reflected light, making it difficult to see). Both are common with multifocal IOLs.

What to Expect During Cataract Surgery
Cataract surgery is typically performed as an outpatient procedure. You will be given anesthesia in the form of eye drops to numb your eye. Typically, you will be fully awake during the surgery. Expect to feel little or no discomfort. Your eye doctor will use a microscope to have a magnified view of your eye. Your natural lens sits in a bag-like structure called the lens capsule. The lens capsule is located just behind the colored part of your eye (iris). The eye doctor makes an incision in the outer surface of the eye, just enough to fit the implant. Through this opening the eye doctor inserts a tiny probe to break up the cataract lens. Your eye doctor then uses a small vacuum-like device to remove the cataract pieces from the eye. Now there will be space for the IOL to be placed in your eye, to replace the natural lens that your eye doctor has just removed. Your doctor will insert the IOL through the same incision. When the surgery is complete, your eye doctor may place a protective patch or shield over your eye.

What to expect after cataract surgery
Right after surgery, you should stay in the recovery area for a short time. Your eye doctor should give you an identification card (typically called implant card) to keep in your wallet. This card shows the type of implant in your eye. Present this card to any eye doctor who examines your eyes after surgery.

Many patients may begin to see better within 1 to 2 days. Some are stable at 10 to 14 days. Some may take 4 to 6 weeks to recover from surgery. Improvements in vision are different for each person.

Call your eye doctor right away if you experience any itching, pain, flashing lights, "floaters", redness, severe headache, nausea/vomiting, light sensitivity, or watery eyes after surgery.

Instructions for post-operative care
You will be allowed to return home on the same day after surgery. Your eye doctor will give you antibiotic eye drops and medicines to speed up healing and to prevent infection. Take all prescribed medicines and apply eye drops as instructed by your eye doctor.

For the first week after surgery, do not go swimming or use a bath or hot tub to help prevent infection. Also try to protect your eye from things like dirt, dust, and strong wind. Keep the eyes protected with sunglasses as much as possible and sleep with your eye shield at night. You should avoid rubbing your eye or putting extra pressure on it. You will be given a date and time for a follow-up visit. It is typically the next day. Your doctor will examine you several more times following your surgery depending on the speed of healing. It may take you some time to get used to new IOL.

Talk to your eye doctor if you have any questions or concerns after surgery.

Undesirable side effects, warnings and residual risk
■ **Visual light phenomena**
After cataract surgery, some patients may notice glare or halos, especially in low light. Halos appear as bright rings around lights, while glare is bright light that can affect clear vision. These effects may be more noticeable when driving at night or looking at artificial lights.
In most cases, these effects improve within a few weeks or months as the brain adapts to the intraocular lens. If they continue after healing, consult your ophthalmologist. Always follow your surgeon's instructions and report any unusual symptoms.

■ **Decentration or deviating diopter power of the lens:**
In rare cases, the implanted lens may move slightly from its position in the eye, even years after surgery. This can cause visual disturbances or glare, which can usually be corrected with a surgical procedure.
Sometimes the lens power may not perfectly match the eye, which can affect vision. This may happen if the lens does not settle exactly as predicted or due to small calculation differences. In most cases, the effect is minor. If the difference is larger, it can usually be corrected with glasses, contact lens, lasik or surgery.

Inform your doctor if you experience:
- a strong pain in your eye
- recurring redness of the eye
- unusual headache or nausea
- considerable vision deterioration
- light flashes, or huge increase of floaters in your eye

These are rare complications that may occur shortly after surgery.

■ **Infections and minor complications**
As with any surgery, there is a small risk of infection after cataract surgery, although serious complications are rare. If an infection occurs, it can usually be treated with prescribed eye drops. You should contact your ophthalmologist if your eye becomes swollen or if you notice any unusual symptoms.

Some minor complications may include:
- Swelling
- Increased pressure in the eye
- Droopy eyelid

These conditions are usually temporary and improve with medication and proper healing. Inform your ophthalmologist immediately if you notice any changes so your recovery can be monitored and treated if needed.

■ **Posterior capsule opacification**
Posterior capsule opacification (PCO) is a common condition that may occur after cataract surgery. It happens when some remaining lens cells grow on the thin membrane behind the implanted lens, causing vision to become cloudy again. Although it is sometimes called a "secondary cataract," it is not a new cataract.

PCO can be easily treated with a quick and painless ND- YAG laser procedure performed by your ophthalmologist. The treatment usually takes only a few minutes, and most patients notice improved vision within a day. If your vision becomes blurry again after surgery, contact your ophthalmologist for further advice.

Patient Card information:
Following your surgery, your healthcare team will provide you with a physical Patient Implant Card. This card contains vital safety information identifying your implanted lens, including the exact product name, model number, dioptric power, batch/serial lot, and unique device identifier (UDI).
Always carry this card with you in your wallet or purse. Show this card to any doctor, ophthalmologist, or hospital staff who treats your eyes in the future.

Magnetic Resonance Compatibility:
The implanted IOL does not contain any metallic components and is safe with no potential for interaction with the Magnetic Resonance (MR) Field. IOL is safer for Magnetic Resonance Imaging (MRI) at a magnetic field strength of 1.5 and 3.0 Tesla.

Expected lifetime and necessary follow-up
The IOL will usually last for a patient's lifetime. Should you however encounter any deterioration in your vision quality, please consult your eye doctor for further investigation and treatment. Your eye doctor will discuss or talk with you about the necessary follow-up intervals based on your medical condition.

Reporting of serious incidents
Any serious incident that occurs in relation to the intraocular lens should be reported to your doctor (in the first instance), to the local representative or to the CAREGROUP SIGHT SOLUTION PRIVATE LIMITED.

About this document:
Your ophthalmologist has performed the cataract surgery and implanted the intraocular lenses (IOL) in your eye. The patient implant card and leaflet have been given to you after the cataract surgery. Please always carry the patient implant card with you to facilitate medical care in case of emergency. The link provided on the implant card will provide you with the latest updates about the IOL.

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